

Patient ID: TMillerPeriod: July 31 to August 30, 2024Report Date: August 31, 2024Coverage: 510/744h (68%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 31 days from Jul 31 to Aug 30

■ HR ■ Breathing

Elevated Breathing Aug 08 to Aug 17 (10d)	Elevated Breathing Aug 20 to Aug 24 (5d)	Breat hing Aug 15 to Aug	High Breathing Aug 22 to Aug 24 (3d)	Low Heart Rate Aug 09 to Aug 15 (7d)	Low Heart Rate Aug 24 to Aug 30 (7d)
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Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: YELLOW

Vital	Avg	Min	Max
Heart Rate	49 bpm	40 bpm	66 bpm
Breathing	18 brpm	11 brpm	44 brpm

Limited data in the final 24h (15/24h); values reflect available readings.

Last 24 hours: one low heart rate episode overnight, otherwise within normal range.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Aug 30	2 AM to 3 AM	1h	1	Elevated Breathing	18	11	44	Check SpO2 and lung sounds; assess for fluid overload or early infection
Aug 30	2 AM to 3 AM	1h	1	Low Heart Rate (brief)	41	40	43	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic

42 episodic events Detected, spanning 3 days total. Conditions: Elevated Breathing, High Breathing, Low Heart Rate. Priority grade: Medium

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
42	78h	Low Heart Rate	1	68%

Major Findings: Sustained low HR (avg 43, min 40)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Aug 15	2 PM to 3 PM	2h	2	High Breathing	32	16	45	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Aug 22	1 PM to 2 PM	2h	2	High Breathing	35	12	55	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Aug 09	11 AM to 12 PM	24h	10	Low Heart Rate	43	40	48	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Aug 24	12 PM to 2 PM	28h	12	Low Heart Rate	43	40	48	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Aug 08	10 AM to 11 AM	6h	6	Elevated Breathing	26	12	53	Check SpO2 and lung sounds; assess for fluid overload or early infection
Aug 20	9 PM to 10 PM	5h	5	Elevated Breathing	25	11	52	Check SpO2 and lung sounds; assess for fluid overload or early infection

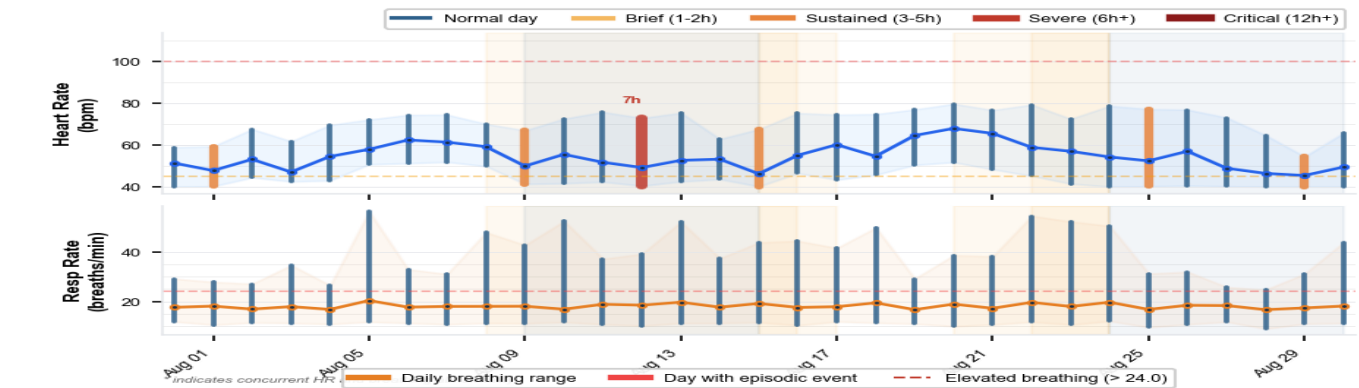
The P5 to P95 heart rate spread was 28 bpm (43 to 71), indicating significant cardiac variability.

Suggested Clinical Review Actions

- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Elevated Breathing (avg 26 brpm, peak 53 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 32 brpm, peak 45 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 6 unstable phases with 28 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

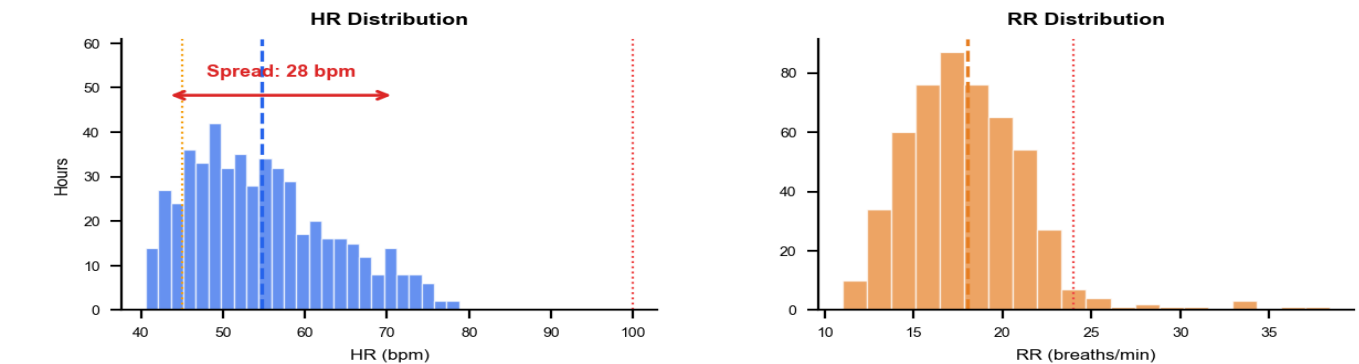
Clinical Pattern Observations:

- 1. **Sustained finding:** 7h continuous Low Heart Rate on Aug 12.
- 2. **High variability:** HR spread 28 bpm (42 to 71).
- 3. **Clustered pattern:** Episodic events on 6 of 31 days (19%).

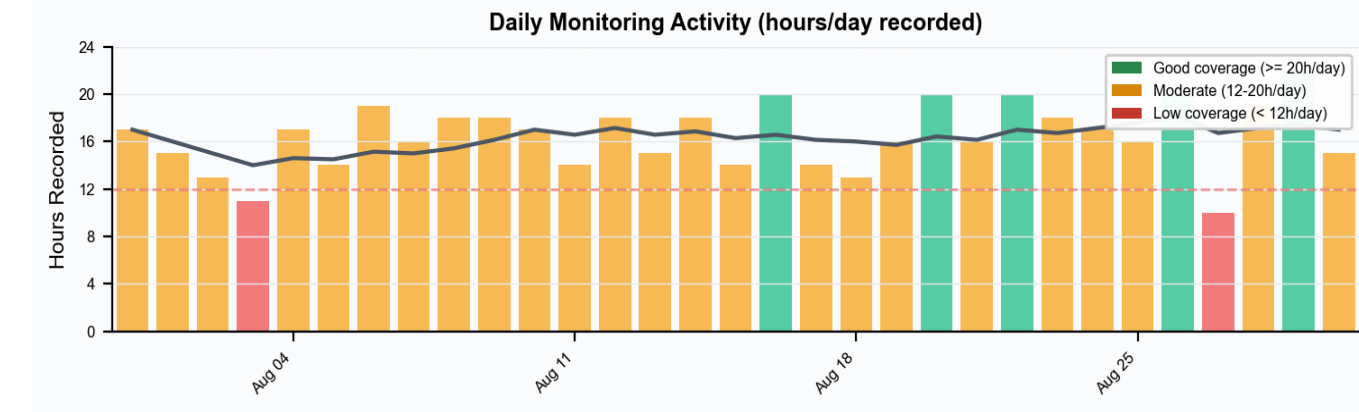


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
 - Low HR < 45 bpm
 - Elevated HR > 95 bpm
 - High HR > 100 bpm
 - Very High HR > 110 bpm
 - Elevated Breathing > 24 brpm
 - High Breathing > 30 brpm
 - Very High Breathing > 40 brpm
- Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.